June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services

Dr. Thomas Keane, M.D., M.B.A.
Assistant Secretary for Technology Policy & National
Coordinator for Health IT
U.S. Department of Health and Human Services

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services

RE: Request for Information: Health Technology Ecosystem (CMS-0042-NC) [RIN 0938-AV68]

Dear Secretary Kennedy, Administrator Oz, and Dr. Keane:

Houston Methodist appreciates the opportunity to submit comments on the Request for Information issued by the Department of Health and Human Services (HHS), the Centers for Medicare & Medicaid Services (CMS), and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC) on behalf of its seven community hospitals and a leading academic medical center serving the Greater Houston area, which includes more than 4,700 physicians and over 30,000 collective employees affiliated with our health care system. In 2024 alone, Houston Methodist provided care to approximately one million Medicare patients.

Houston Methodist is committed to the safety of our patients and the communities we serve. Houston Methodist Hospital, the system's flagship, has consistently been listed among *U.S. News & World Report's* best hospitals and the top hospital in Texas, which is supported by academic institute, a comprehensive residency program, a global business division, numerous physician practices and several free-standing emergency rooms and outpatient facilities, and a leading accountable care organization that works with the system to improve care for the people of Houston and surrounding areas.[1]

[1] https://www.houstonmethodist.org/about-us/coordinated-care/

Houston Methodist is eager to work with CMS, HHS, and ONC to align regulatory frameworks and payment models with technologies that enhance clinical outcomes, expand equitable access, and advance the broader goals of the Making America Healthy Again (MAHA) initiative.

Since its launch in 2017, Houston Methodist Coordinated Care (HMCC), our accountable care organization (ACO), has consistently generated savings by outperforming CMS benchmarks. These results stem from strategic care coordination efforts focused on reducing avoidable admissions, preventing readmissions, and minimizing unnecessary ED visits. Based on this experience, we offer the following comments and recommendations for both the provider and value-based care organization sections of the RFI.

1. Digital Health Applications and Provider Adoption (PR-1 to PR-4)

PR-1. Encouraging Provider Use of Digital Health Products

Houston Methodist strongly supports efforts to expand access to and adoption of approved digital health products for providers. Remote Patient Monitoring (RPM) has evolved from a tool primarily used in intensive care units (ICUs) to a widely adopted solution across acute care settings. RPM offers continuous surveillance and early detection of changes in patient conditions. The newest generation of wearable medical devices provide real-time, continuous vital sign data for all hospitalized patients, enhancing clinical responsiveness.

In April 2023, Houston Methodist launched RPM initiative using the BioButton®[2]across all acute care units. Initially deployed at our flagship hospital, the program has expanded to over 2,600 non-ICU beds. The BioButton is a small, battery-operated device that sticks to the patient's chest upon admission, continuously monitoring heart rate, respiratory rate, and skin temperature. The data feeds into a central dashboard, where AI-driven analytics identify concerning trends and notify the care team in real time.

This program has already yielded improved patient outcomes, enhanced safety, and reduced the workload for bedside nurses. Based on these results, we are exploring outpatient applications to support post-discharge recovery. To enable broader adoption of RPM technologies, we urge CMS and ONC to eliminate reimbursement barriers for RPM devices and promote integration of standardized device data into EHR systems.

PR-1a. Current Obstacles

Despite their clinical value, RPM devices often lack adequate Medicare coverage, making them unaffordable for many providers. While Houston Methodist covers the annual subscription cost for the BioButton® without separate Medicare payment, most providers lack the financial flexibility to

[2] BioButton® is a trademark of BioIntelliSense, Inc.

adopt such technology at scale. Additionally, RPM implementation requires dedicated staff to respond to alerts and manage technical issues—resources that many organizations cannot spare.

Compounding these challenges, current fee-for-service payment models prioritize time-based billing over long-term disease control, misaligning incentives and undervaluing outcomes. To address this, we urge CMS to:

- Eliminate arbitrary requirements such as the 16-day data threshold for RPM and Remote Therapeutic Monitoring (RTM), which often lack clinical relevance and limit appropriate use.
- Adopt the American Medical Association's CPT Panel recommendation to establish new supply codes that differentiate between 2–15 days and 16+ days of device use and ensure fair and sustainable reimbursement for both.
- Shift RPM reimbursement to reflect the full scope of care—device provisioning, clinical integration, and patient outcomes—rather than minutes of monitoring or interaction time.

Since the introduction of RPM and RTM codes, reimbursement cuts under the Medicare Physician Fee Schedule have jeopardized program sustainability, particularly for providers in underserved or rural areas. RPM should be treated not as an optional add-on, but as an essential infrastructure for managing chronic conditions, facilitating transitional care, and supporting early intervention. We respectfully urge CMS to halt further payment reductions and ensure rates reflect the clinical and operational demands of high-quality RPM services.

PR-1b and PR-1c. Information and Responsibilities When Using Digital Tools

Providers have a responsibility to ensure that patients understand the goals of any digital health intervention, how their health data will be used, and the critical role they play in co-managing their health. Houston Methodist informs every patient that is admitted about the use of the BioButton® and the associated AI-algorithms used to monitor vital trends.

PR-2. Obstacles in Physician Workflows

Houston Methodist recognizes several key obstacles that hinder providers from developing, deploying, and utilizing digital health tools including cost constraints, limitations on telehealth use, and challenges related to reimbursement and coverage.

Addressing Cost Barriers

Cost remains a significant barrier to broader adoption of digital and AI-enabled tools, particularly among smaller practices. To address this challenge, CMS should consider shifting away from time-based reimbursement models and move towards rewarding improved clinical outcomes. Though Houston Methodist and other large systems have been able to overcome this cost barrier, may smaller practices and community-based providers cannot.

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Support for Permanent Telehealth Flexibilities

While Houston Methodist supports CMS's decision to allow direct supervision via telehealth available through CY2025, we urge CMS to make this permanent across all scenarios. Clinical teams are best positioned to determine when telehealth-based direct supervision is appropriate, based on the specific needs and circumstances of their patients. Making this flexibility permanent would support innovation and the sustainable integration of virtual care into clinical workflows.

Coverage and Reimbursement for Digital Health Tools

Uncertainty regarding the reimbursement of digital health tools is another notable barrier. CMS should review coverage pathways and revise reimbursement frameworks to reflect the innovative and diverse nature of digital health solutions and products. A lack of clear guidance about whether and how digital solutions—such as predictive analytics platforms, decision support tools, and behavior-modification technologies—can be reimbursed under Medicare or Medicaid leads to underutilization of otherwise valuable care management tools.

Houston Methodist encourages CMS to offer specific guidance for providers and Medicare Advantage plans regarding coverage of emerging technologies, including those aimed at non-mental health indications (e.g., cardiology, pulmonology, neurology). Additionally, CMS should consider permitting these tools to be reimbursed under a temporary transitional coverage mechanism while more permanent policies are developed. To this end, CMS could pilot a one- to two-year coverage window to allow new digital tools to gain experience, produce outcomes data, and support the case for longer-term coverage. This would offer valuable flexibility and promote innovation while preserving program integrity.

PR-3 and PR-4. Importance of EHR Data Accessibility and Third-Party App Access to Administrative Workflows

Effective remote patient monitoring systems depend on clinicians having real-time access to comprehensive health data and seamless integration into the Electronic Health Record (EHR). It's important that any new device or digital health tool that is designed to assist physician workflows provide seamless data exchange between the device and the EHR system to support patient monitoring.

Similarly, enabling third party applications to support administrative workflows requires more than just access to patient data, it requires integration into full operational functions such as scheduling and communication across the system. To facilitate this and make this easier for providers, CMS and ONC should strengthen support for cross-platform functionality and enhance interoperability between care outreach platforms and payer administrative systems.

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2. Digital Health In Value-Based Care Models (VB-1 to VB-4)

VB-1. Incentives to Drive ACO/MSSP Adoption

One of the current challenges in value-based care (VBC) programs is the CMS requirement that ACO's must aggregate all data from various EHR systems for every patient, regardless of payer. While this mandate aims to promote comprehensive care coordination, it places a considerable burden on participating entities. Many small, rural, or resource-constrained organizations —as well as independent primary care physicians and solo practitioners—are disproportionately impacted due to the substantial investments required for data integration technology, IT infrastructure, and training. These burdens can create a barrier to entry for physician practices that may otherwise be well-positioned to participate in ACOs but lack the technical and financial capacity to comply.

The lack of interoperability among EHR platforms further compounds the issue, creating inefficiencies and increasing administrative burdens. To mitigate these challenges, we recommend that CMS provide targeted financial and technical support to both ACOs and affiliated providers to build the necessary infrastructure for data integration. This could include grant funding or incentive programs modeled after the HITECH Act to support investment in interoperable health IT systems, staff training, and vendor coordination. In parallel, CMS should work with the Office of the National Coordinator for Health IT (ONC) to enforce and accelerate the adoption of standardized interoperability frameworks, such as FHIR-based APIs and USCDI-aligned data elements, across certified EHR vendors. These standards are critical to enabling seamless data exchange across platforms and reducing the complexity and cost of multi-payer data aggregation. Without such federal coordination and vendor accountability, the burden of integration will continue to fall disproportionately on ACOs, undermining the goals of value-based care and widening disparities in participation.

VB-2. Integrating Technologies into APMs

HMCC's strategy to generate savings focuses on preventing avoidable high-cost patient events by using predictive analytics. These capabilities allow our team to identify and flag high-risk patients and deploy case management, nurse outreach programs, or education initiatives to prevent admission, readmission, or an unnecessary visit. Integrating predictive analytics and other digital health technologies into Alternative Payment Model (APM) workflows beyond data collection is crucial as predictive analytics or other AI would not be able be as effective in flagging non-adherence.

However, the utility of such tools is limited when reimbursement fails to match the operational realities of implementation. We encourage CMS to ensure digital monitoring tools that support APM success are sufficiently reimbursed. CMS should consider reimbursement models that tie monthly payments to outcomes or chronic condition control rates, as opposed to volume-based service

codes. CMS should also ensure that devices used in these models are eligible for coverage whether they are classified under legacy benefit categories such as DME, or as incident-to supplies.

VB-3. Essential Health IT Capabilities

To support effective value-based care arrangements, CMS and ONC should prioritize platforms capable of risk stratification as well as actionable care planning. These systems must help clinical teams identify patients in need of additional services and guide interventions across the care continuum.

VB-4. Essential Data Types

While the BioButton® offers continuous vital sign monitoring, identifying high-risk patients requires additional data, including claims and encounter data, and social determinants of health (SDOH). Access to this broader data enables our case managers and nurse outreach teams to deliver more targeted interventions, improving care quality while reducing costs.

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We appreciate your consideration of our comments. If you have any questions or would like additional information, please feel free to contact me at dmorales@houstonmethodist.org.

Sincerely,

/s/

Daniel Morales

Vice President, Government Affairs